BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker. EMA supplementary follow-up: cardiac assessment every 6 months after trastuzumab until 24 months; after anthracyclines in early disease, yearly to5 years from the last trastuzumab dose, or longer if function falls.

Check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Blood tests	FBC, kidney and liver profile	Review before chemotherapy cycles	Check again at any phase change	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart	Baseline before AC	Reassess before starting trastuzumab	Every 12 weeks during trastuzumab	Continue the same12-week clock after paclitaxel	After trastuzumab: follow EMA/ESC and cardiac risk plan
ECG	Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging	Record baseline extent as appropriate	Clinical response / symptoms review	If before surgery: plan response review	Arrange selected surgery / next treatment	Planned breast imaging; no routine CT/PET
Markers / bone health	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/bone-risk indication	Follow the endocrine and bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00432 v5 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

AC → weekly paclitaxel + weekly trastuzumab: treatment and checks

HER2-positive neoadjuvant or adjuvant high-risk breast cancer

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00432	Agree aim, indication and medicines	AC → weekly paclitaxel + weekly trastuzumab starts	Change from AC to paclitaxel; select new support script	Stop paclitaxel → trastuzumab alone	Complete one total year of trastuzumab
Surgery EARLY	Adjuvant: surgery already completed	Neoadjuvant: chemotherapy first	Review response / surgical plan	Surgery after neoadjuvant therapy, if applicable	Pathology and recovery guide next steps
Radiotherapy EARLY	Assess indication	Usually after cytotoxic chemotherapy	Arrange sequential timing	Radiotherapy if indicated; HER2 may overlap by MDT plan	Review effects / follow-up
Blood tests NCCP 00432	FBC, kidney and liver profile	Review before chemotherapy cycles	Check again at any phase change	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00432 · ESC	Baseline before AC	Reassess before starting trastuzumab	Every 12 weeks during trastuzumab	Continue the same12-week clock after paclitaxel	After trastuzumab: follow EMA/ESC and cardiac risk plan
ECG NCCP 00432	Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging NCCP 00432	Record baseline extent as appropriate	Clinical response / symptoms review	If before surgery: plan response review	Arrange selected surgery / next treatment	Planned breast imaging; no routine CT/PET
Markers / bone health NCCP 00432	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/bone-risk indication	Follow the endocrine and bone-health plan
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1 Before first dose

Baseline

Baseline FBC, kidney and liver tests; review before each AC cycle and each paclitaxel administration under the regimen and local testing plan.

Ask / act: Confirm tests and consent before prescribing.

Sources: NCCP 00432

2 Heart-monitoring plan

Baseline → cycles → after treatment

ESC supplementary guidance: baseline echo before anthracycline treatment. High/very-high cardiac risk: echo every two anthracycline cycles (for example before cycles 3 and 5 when continuing), and within 3 months after completion. All adults: echo within 12 months after completion. Other repeat tests depend on risk, cumulative exposure and symptoms. Reassess before the first trastuzumab, then echo/MUGA every 12 weeks during trastuzumab. In an uninterrupted weekly taxane phase,12 weeks is near its end; in21-day continuation this is about every 4 cycles. Keep the original clock through the transition. Earlier assessment is needed for concern.

Ask / act: Name the team responsible for booking and reviewing results.

Sources: NCCP 00432 · ESC

3 Treatment-phase review

Before each transition

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

Ask / act: Confirm active medicines, support script and the next test checkpoint.

Sources: NCCP 00432

4 Response and follow-up

Planned review points

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Ask / act: Review symptoms and imaging together; do not change treatment on tumour markers alone.

Sources: NCCP 00432

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-23 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.17